HELP DESK RESOLUTION PORTFOLIO

Learner name	[Enter name]
Completion date	November 14, 2026
Target role(s)	Healthcare IT Support Specialist / Service Desk Analyst / Desktop Support Technician / EHR Support Specialist
Portfolio version	v1.0

SECTION 1 — COVER SUMMARY

This portfolio documents three Tier 1 help desk tickets worked end to end in a simulated healthcare environment, one in each of three domains: Identity and Access (new-user provisioning for a Med/Surg registered nurse across Microsoft 365, Intune, and the Entra ID group that feeds Epic), Data Privacy / Urgent Escalation (containment and Compliance Officer warm-transfer of a suspected PHI exposure in a Teams channel), and End-User Enablement / AI (a reusable Microsoft Copilot prompt framework with coaching notes for a research team).

Four disciplines run through all three tickets: PHI-plain-text discipline, the standard warm-transfer summary format, structure-not-output discipline, and Tier 1 boundary respect. Each ticket section leads with the actions taken and the evidence captured, then the escalation or handoff record, then a short rationale. All scenarios, timestamps, and evidence references come from capstone simulations, not a production environment.

SECTION 2 — TICKET 1 RESOLUTION | IDENTITY AND ACCESS

Technical notes

Ticket TKT-001 | Standard priority | New RN – Med/Surg, requested by the Nursing Manager. Worked Friday, 3:58 PM to 5:00 PM.

Microsoft 365: Created the account with display name and UPN in the organization's firstname.lastname format and assigned the Microsoft 365 E3 Healthcare license (4:05 PM). Delivered the temporary credential to the requesting Nursing Manager through the secure onboarding portal, not to the new hire and not in the ticket (4:10 PM). Mailbox active 9 minutes after licensing, inside the 15-minute window (4:14 PM).

Intune: Located the assigned workstation and confirmed Windows Autopilot enrollment was complete (4:22 PM). Assigned the existing Med/Surg Workstation Compliance Profile from the role-to-profile mapping table; device state Compliant (4:31 PM).

Epic / Entra ID: Verified the existing RN-Med-Surg group against the integration matrix, then added the account (4:38 PM). Downstream sync to the Epic provider security group confirmed in 8 minutes, inside the 10-minute window (4:46 PM).

Evidence: Screenshot references recorded for each sub-step with timestamps; no credential values captured.

Documentation and next-shift handoff

Completed: Microsoft 365 account, license, mailbox, and credential delivery (4:05–4:14 PM); Intune profile assigned and device Compliant (4:31 PM); RN-Med-Surg membership added and Epic sync confirmed (4:38–4:46 PM). Handoff note written 5:00 PM, ahead of the Sunday evening deadline.

Open for incoming shift: Verify Monday first-time sign-in for New RN – Med/Surg at shift start: credential change, mailbox, compliant workstation, Epic access.

Anomalies: None observed. No sync was left pending.

Escalation communications

No escalation trigger fired. The ticket completed within the defined Tier 1 scope, so no warm-transfer summary was required for TKT-001.

Rationale

Tier 1 scope on this ticket was limited to creating the account to the documented standard, assigning an existing compliance profile, and adding an approved account to an existing group; no roles, profiles, or group definitions were created or changed. Escalation would have been triggered by an unclear UPN format, no available license seat, a mailbox or Autopilot delay beyond

15 minutes, a profile that did not apply cleanly, a missing or mismatched group, or a sync beyond 10 minutes, routed to the Identity Administrator, Endpoint Administrator, or Epic Analyst team. None occurred, so the resolution stayed within scope.

SECTION 3 — TICKET 2 RESOLUTION | DATA PRIVACY / URGENT ESCALATION

Technical notes

Ticket INC-00472 | Urgent priority | Reporter: Attending Physician | Patient reference: Patient A.

10:47 AM: Content that appeared to be a patient chart note excerpt was posted to the clinic-announcements Teams channel (500+ members) in place of a schedule-change notice. Deleted by the Attending Physician at approximately 10:47:30 AM.

10:48 AM: Call received and ticket assigned. Reporter asked not to re-open, re-paste, or describe the content.

10:49 AM: Confirmed the message was no longer visible in the channel.

10:51 AM: Evidence preserved: audit and compliance records left unaltered; no purge, retention, export, or content-recovery action taken. Interaction evidence: at least three recorded reactions before deletion; total viewership unknown and not inferred.

10:53–10:56 AM: HIPAA/PHI incident response checklist completed. Outcome: outside documented Tier 1 SOP. No channel members contacted; no channel restriction or membership change made.

Warm-transfer summary — standard core fields

To	Compliance Officer (designated privacy/compliance function)
From	Tier 1 Support Technician
Ticket Number	INC-00472
Timestamp	Warm-transfer sent 10:58 AM. Incident: posted 10:47 AM; deleted approximately 10:47:30 AM; reported 10:48 AM.
Clinician Description	Attending Physician
Component / Incident Type	Suspected PHI exposure in Teams channel
Signal / Outcome	What appeared to be a patient chart note excerpt (Patient A) was posted to the clinic-announcements channel and deleted within approximately 30 seconds. At least three reactions were recorded before deletion; total viewership unknown.
Action Required	Formal breach risk assessment and determination of required next steps

Ticket 2 supplemental fields

Checklist Outcome: Outside documented SOP — Compliance Officer determination required.

SOP-Fit Assessment: Not SOP-fit. Suspected unauthorized PHI disclosure via a general communication channel.

Ambiguities Flagged: Breach status determination; patient notification requirements; channel-lockdown decision.

Clinician communication log

11:00 AM, by phone, to the Attending Physician:

Containment status: Message no longer visible in the channel; evidence preserved.

Escalation status: Warm-transfer to the Compliance Officer in progress under INC-00472.

Next-step ownership: The Compliance Officer will follow up with her directly; nothing further for her to do in the channel.

Not said: No opinion on breach status, disciplinary outcome, or organizational response, and no promises about outcomes. Call ended 11:02 AM; ticket status set to escalated, pending determination at 11:04 AM.

Rationale

Tier 1's role in a suspected PHI exposure is to confirm containment, preserve evidence, document the incident without reproducing the content, and hand off. Whether a breach occurred, whether patients must be notified, and whether the channel should be restricted are formal determinations that belong to the authorized privacy/compliance function, so those three questions were flagged as ambiguities and routed, not answered.

SECTION 4 — TICKET 3 RESOLUTION | END-USER ENABLEMENT / AI

Program-defined RCCOF framework

Ticket INC-00531 | Standard priority. Scope: non-PHI, publicly available external research papers only. RCCOF is the five-part prompting framework used in this program; it is not presented as an official Microsoft-standard framework.

Role	Clinical research summarization assistant producing an accurate, neutral summary of one published external paper for an internal review meeting; not a clinical advisor, peer reviewer, or strategy advisor.
Context	One named, publicly available external paper (title, authors, source, year, DOI) supplied as the only source; explicitly not an internal document, draft, unpublished data, or patient information.
Constraints	Use only the named paper: no content from other open documents or tabs, no editorial commentary, no institutional strategy content, no recommendations of Copilot's own, no gap-filling. These scope the task; they are not a security boundary.
Output format	Fixed headings in 400 words or fewer: citation, objective, methods, key findings with page or section references, stated limitations, authors' conclusions, a source-check line, and a final line requiring human verification.
Follow-up verification	Three human checks with the paper open: citation accuracy; cross-document content (intended-source grounding); clinical-recommendation fidelity. A summary is not shared until all three pass.

Coaching notes

Components in plain language: Role gives Copilot one job. Context names the one paper that counts. Constraints list what to leave out. Output format makes every summary comparable and traceable to a page. Follow-up verification is a person checking the result.

Source scoping: Before prompting, close other documents and tabs, start a new chat, and attach only the public paper.

Worked examples (illustrative, non-PHI): a clinical outcomes study, where every number is checked against the tables; a public health guideline, where recommendations keep the guideline's own wording; a conference proceeding, where "Not stated in the paper" is the correct answer for gaps.

What protects data: Prompt wording alone does not. Protection comes from source scoping, the organization's data-protection controls, and human verification together.

Common pitfalls: Using the framework with non-eligible content; skipping verification under time pressure; ignoring a cross-document content warning in the output.

Delivery communication

Sent to the requesting Medical Director (Internal Medicine Research), under 150 words. It confirmed delivery of the RCCOF framework and coaching notes, stated that the three verification checks are essential and should not be skipped, stated that the framework reduces risk only when those checks are completed, and routed organizational AI-use policy questions to the Information Governance team.

Rationale

I delivered a prompt structure with placeholders, not Copilot output. The framework was not tested on any institutional draft content, no internal excerpts appear in the framework or coaching notes, and the worked examples are invented public paper types. The claim is limited to what the structure can support: it reduces risk when the human verification steps are followed, and prompting alone does not prevent data leakage.

SECTION 5 — CROSS-TICKET SYNTHESIS

Ticket / Domain	Primary artifact(s)	Capstone discipline demonstrated
Ticket 1 — Identity and Access	Next-shift handoff; escalation evidence if applicable	Privacy/minimum-necessary documentation; Tier 1 boundary respect
Ticket 2 — Data Privacy / Urgent Escalation	Containment notes; warm-transfer summary; clinician communication	Warm-transfer discipline; privacy/redaction discipline; Tier 1 boundary respect
Ticket 3 — End-User Enablement / AI	RCCOF framework; coaching notes; delivery communication	Structure-not-output discipline; Tier 1 boundary respect

Across three unrelated domains the same four disciplines decided what I did and what I handed off. PHI-plain-text discipline shows in the Ticket 1 next-shift handoff documentation, where the open item refers to “New RN – Med/Surg” and the temporary credential never enters the ticket, and in the Ticket 2 Compliance Officer warm-transfer summary, which describes the incident only as what appeared to be a patient chart note excerpt relating to Patient A, reported by the Attending Physician.

The warm-transfer summary format is the same in every ticket: receiving team, ticket number, exact timestamp, role-label description, observed signal, action required. In Ticket 2 it carried the incident timestamps, the checklist outcome, the not-SOP-fit assessment, and three flagged ambiguities to the Compliance Officer. In Ticket 1 the same format was ready for the Identity Administrator, Endpoint Administrator, or Epic Analyst team, and the record shows that no trigger fired. Structure-not-output discipline is clearest in the Ticket 3 RCCOF framework structure: Role, Context, Constraints, Output format, and Follow-up verification were delivered as placeholders and checks, never tested on institutional content, with the claim limited to reducing risk.

Tier 1 boundary respect links all three. I added an account to an existing group without changing its definition (Ticket 1), contained and preserved evidence without assessing breach status or restricting the channel (Ticket 2), and routed AI-use policy questions to Information Governance instead of answering them (Ticket 3). Each artifact records what I did not do as clearly as what I did.

SECTION 6 — CAREER READINESS SIGNALS

This portfolio is evidence for entry-level roles such as Healthcare IT Support Specialist, Service Desk Analyst, Desktop Support Technician, and EHR Support Specialist. Following a documented provisioning procedure across Microsoft 365, Intune, and Entra ID, and timestamping each step, is relevant to service desk and desktop support work. Verifying an Entra ID-to-Epic group mapping and writing a next-shift handoff is relevant to EHR support. Containing a privacy incident, documenting it without PHI, and escalating in a standard format is relevant to any healthcare support desk, as is coaching clinical and research users on a Copilot prompt structure without overstating what it protects. The work was done in simulation with supplied templates and reference sets, so it demonstrates procedure, documentation, and judgment about scope, not production administration experience.

FINAL REVIEW — SCANNABILITY + PRIVACY / SENSITIVE-DATA AUDIT

Portfolio scannability

☒ All three ticket category labels are immediately visible in the section headings.

☒ A reviewer can quickly identify the recurring discipline patterns and the artifacts that demonstrate them.

☒ Technical notes lead with actions/evidence rather than long background paragraphs.

☒ The document can be navigated efficiently without reading every sentence to understand the work completed.

Privacy and sensitive-data audit

☒ No real patient PHI or patient identifiers appear anywhere in the portfolio or embedded evidence.

☒ No unnecessary clinician/workforce identifiers appear in external-facing portfolio content; role labels are used where individual identity is not needed.

☒ No credentials, temporary passwords, authentication secrets, tokens, or other security-sensitive values appear in screenshots, ticket excerpts, or artifact descriptions.

☒ No verbatim patient chart content or other clinical source material appears in the Ticket 2 section.

☒ No sensitive institutional strategy, draft research, clinical-trial, patent-adjacent, or other restricted internal content appears in the Ticket 3 section or examples.

☒ Screenshots and embedded evidence were reviewed separately to confirm that identifiers or sensitive values are not visible (no screenshots are embedded in this portfolio).